DOCUMENT 2 OF 2 · GO-TO-MARKET

The 60-Day Founding Clinic Pilot

Your pilot plan for UAE & USA dermatology, cosmetic dental and aesthetic clinic owners: offer, pricing (pilot and after), lead lists, Instagram + LinkedIn + email outreach with scripts, sales calls, delivery, metrics, competitor positioning and compliance.

Prepared for Dr. Azka · Pilot window: November–December 2026 · Prepared 31 October 2026

1. The pilot on one page

5	60	2	3
founding clinics (max)	days per clinic	markets: UAE + USA	specialties

Element	Decision
Who	Doctor-owners (or the medical director with budget control) of **dermatology, cosmetic dentistry and aesthetic medicine** clinics. Already established (ideally $10k+/month, a front desk, high-ticket treatments). **UAE:** Dubai first, then Abu Dhabi. **USA:** pick 3–4 high-aesthetic metros (e.g. Miami, Dallas, Houston, Los Angeles/Orange County).
What you sell	Not 'content' and not 'AI'. You sell **more booked patients + less patient leakage + less manual work**, delivered by a doctor.
Pilot offer	**Founding Clinic Partner:** 40 doctor-reviewed videos/month for 1 doctor + 3 core AI systems + 1 database reactivation campaign, at a founding price, for 60 days. In return you get case-study rights.
Front-end hook	Free **'Patient Leak Audit'**: a 2–3 min personalised Loom covering their content, inquiry response speed and reactivation gap.
Channels	Instagram (warm, doctor-to-doctor) · LinkedIn (owners, practice managers, US group practices) · Cold email (scale) · Your own content (inbound: 'comment LEAK').
Pilot goal	Sign 3–5 founding clinics by early December. Produce 2+ documented case studies with a baseline, timeframe and attribution. Convert at least 60% to full price after 60 days.

An honest timeline check on your Dec 31 goal ($10–12k/month)

Founding clients at pilot prices give roughly $5–7k MRR by December. To reach $10–12k by Dec 31 you'd need the pilots *plus* 2–3 clients at standard pricing, which is hard before you have case studies. A more realistic target: **$10–12k by end of February 2027**, when pilots convert to full price. The section 6 pricing reset makes your later goals much easier: $50k/month needs about 15–17 clients at a $3k average, instead of 25–40.

2. Ideal pilot clinic: qualify before you pitch

Score every clinic out of 10 and spend most of your personalised time on 7+ (Tier A).

Signal	Points	How to check
Doctor is owner / founder / medical director with budget control	2	Website 'About', LinkedIn, licence registry
High-ticket menu (veneers, implants, aligners, lasers, injectables, Morpheus8, hair, body)	2	Website treatment menu
Instagram is active but weak (generic flyers, no doctor on camera, <4 Reels/month, no clear CTA)	2	Profile scan, 2 minutes
Slow or no response to a genuine after-hours inquiry (DM / WhatsApp / form)	2	Your leak test (see section 4)
2+ providers, or a front desk (manual follow-up is already a bottleneck)	1	Website team page
Strong reputation (≈4.5★+, 100+ Google reviews), so they have demand worth protecting	1	Google Maps

Disqualify: brand-new clinics struggling for their first patients, chains with in-house marketing teams (unless you're selling to a single branch's doctor), and clinics that want ads-only or 'just posting'.

Where to find them

Market	Sources	Search terms / filters
UAE	Google Maps; Instagram location tags and hashtags; LinkedIn Sales Navigator; DHA / DoH professional registers to verify that licences and roles are real	'dermatology clinic Jumeirah / Al Wasl / DHCC', 'cosmetic dentist Dubai Marina', 'aesthetic clinic Abu Dhabi'; #dubaidermatologist #dubaidentist #dubaiaesthetics
USA	Google Maps; AAD 'Find a Dermatologist'; AACD member directory (cosmetic dentists); Apollo / Clay for verified emails; LinkedIn Sales Navigator	Title: Owner, Founder, Medical Director, DDS/MD; Industry: Medical Practices; Geography: chosen metros; company size 2–50

List size for the pilot: 400 clinics (≈200 UAE, ≈200 USA). About 80 Tier A get heavy personalisation and a Loom; about 320 Tier B get light personalisation. Track everything in one CRM. Use GoHighLevel or similar, since you'll need it for the AI systems anyway.

3. Week 0 setup (do this first, Nov 1–7)

Task	Detail
Your profiles (5-second rule)	**Instagram bio:** L1 'Dr. Azka · Doctor helping clinics get more patients' · L2 'Content that books + AI that never misses an inquiry' · L3 'Dermatology · Cosmetic Dental · Aesthetics | UAE · USA' · L4 '↓ Free Patient Leak Audit'. Name field: 'Dr. Azka | Clinic Growth & AI' (searchable). **LinkedIn headline:** 'Doctor helping dermatology, dental & aesthetic clinics turn content into booked patients, with AI follow-up | UAE · USA'.
Proof assets	3 sample scripts per specialty (9 total, pulled from Document 1), 1 demo video of your AI receptionist/DM bot answering a clinic inquiry, a one-page offer PDF, and a sample 'Clinic Growth Report' with dummy data.
Email infrastructure	Buy 2–3 secondary domains (never cold-email from your main domain). Set up 2 inboxes per domain with SPF/DKIM/DMARC and start warm-up **now**. Warm-up takes about 2 weeks, so email goes live in week 3 while Instagram and LinkedIn run from week 1. Tools: Instantly or Smartlead + Apollo.
Booking + CRM	Calendar link with 3 qualifying questions (specialty, city, monthly new patients). Pipeline stages: Lead → Engaged → Audit sent → Call booked → Proposal → Won/Lost.
Legal pack	Pilot agreement (60 days, payment terms, case-study + testimonial permission, early exit). Data-processing terms. For US clients, AI/CRM tools that will sign a **HIPAA Business Associate Agreement**. A patient-consent checklist for messaging and testimonials. Get a lawyer to review once; it's cheap insurance.
Your own content	Start the 30-day sequence from Document 1 (1–2 posts/day). The 'I messaged 10 clinics at 9 pm' series and the 'comment LEAK' CTA feed your pipeline directly.

4. Outreach system: Instagram, LinkedIn, Email

The 'Patient Leak Test' (your unfair advantage in outreach)

Before contacting a Tier A clinic, send a **genuine, general inquiry** to their Instagram/WhatsApp/web form after hours (e.g. 'Hi, do you offer veneers consultations and roughly what's the process?'). Note the time and how long the reply takes. Then you open with *their* data, not a generic pitch. **Rules:** ask a genuine question, never book or hold a fake appointment, and be transparent when you mention it ('I tested your inbox as a patient would').

14-day multichannel sequence (per Tier A clinic)

Day	Channel	Action
−3 to 0	Instagram	Follow the clinic + the doctor. Leave 2 genuine comments on recent posts (clinical, specific, never 'great post!'). Run the leak test.
1	LinkedIn	Connection request with a note (script L1). No pitch.
2	Instagram DM	Opener (script I1): doctor to doctor, one question.
3	Email	Email 1: the leak observation + offer of the audit video.
5	Instagram / Email	Send the 2–3 min Loom audit (to whoever replied, or by email).
7	LinkedIn	Message L2: share one relevant post of yours, no ask.
8	Email	Email 2: a free script written for their clinic.
11	Email / DM	Email 3: the founding-partner offer (5 spots).
14	Email	Email 4: polite close-out. Move to the monthly nurture list.

Daily volume (solo, sustainable)

Activity	Per day	Per week	Note
Instagram genuine engagement	30–40 comments/likes	~200	Warms DMs. Keep it human; avoid automation tools on IG.
Instagram DMs	20	~100	Only to clinics you've engaged with first.
LinkedIn connections	15–20	~100	Stay within LinkedIn's weekly invite limits.
Cold email (after warm-up)	30–40 per inbox	~600–800	Plain text, no images, 1 link max, opt-out line.
Loom audits	3–5	~20	Tier A only. Your highest-converting asset.

> **Planning assumptions (replace them with your own data by week 2)**
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> About 900 touches/week → roughly 25–40 replies → 6–10 audit calls → 1–2 founding clients/week. Cold email typically gets the lowest reply rate; warm Instagram DMs and Looms usually get the highest. Track by channel and move effort to whichever channel books calls.

Scripts

Value-per-line rule for outreach: every line must contain *their* name, *their* data, a specific observation or a single question. If a line could be sent to any clinic, delete it.

Instagram

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I1 — Opener (after engaging)
Hi Dr. [Name], fellow doctor here 👋 Your reel on [topic] was great, especially [specific
point].
Quick question: are your Reels bringing in consultations right now, or mostly views?

I2 — If they reply
That's really common. The content is usually fine; the leak is after the inquiry.
I actually messaged your clinic's IG on [day] at [time] asking about [treatment], and the reply
came [X hours] later.
I recorded a 2-min video showing where I'd fix it (content + inquiry follow-up). Want me to
send it here?

I3 — No reply after 4 days
No worries if now's busy, Dr. [Name]. I'll just leave the 2-min audit here in case it's useful:
[Loom link]. No pitch in it.
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LinkedIn

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L1 — Connection note (under 300 characters)
Dr. [Name], I'm a doctor working with dermatology & cosmetic dental clinics in [Dubai / the US]
on turning content into booked patients, with AI follow-up so no inquiry is missed. Would love
to connect.

L2 — After accepting (day 7)
Thanks for connecting, Dr. [Name]. I posted this breakdown of where [specialty] clinics lose
patients between the DM and the booking: [link]. Curious whether it matches what you see at
[Clinic].

L3 — Practice manager / marketing lead version (US group practices)
Hi [Name], I help [specialty] practices track which content actually produces consultations,
and stop after-hours inquiries going cold. Happy to send a 2-min audit of [Clinic]'s patient
journey if useful.
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Email

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Email 1 — Subject: [Clinic], 9pm inquiry
Hi Dr. [Last name],
I'm Dr. Azka, a doctor who now helps dermatology and cosmetic dental clinics turn Instagram
into booked consultations.
On [day] at [time] I messaged [Clinic]'s Instagram as a patient would, asking about
[treatment]. The reply came [X hours] later.
Not a criticism. It's the most common leak I find: the content works, but the inquiry cools
before anyone answers.
I recorded a 2-minute video showing:
1) the 3 posts on your profile closest to bringing patients,
2) where inquiries are slipping,
3) the first fix I'd install.
Worth sending over?
Dr. Azka
[Opt-out line + business address, required for US cold email]

Email 2 (day 8) — Subject: wrote this for [Clinic]
Dr. [Last name], I wrote one script for your [treatment] based on what patients ask most.
Here's the hook and outline:
"[Hook from Document 1, adapted]"
Every script we make is reviewed by a doctor and fact-checked. That's the part generic agencies
skip.
Want the other two?

Email 3 (day 11) — Subject: 5 founding clinics
Dr. [Last name], I'm taking on 5 founding clinics (UAE + US) for a 60-day pilot:
• 40 doctor-reviewed videos/month (strategy, scripting, editing; you film in two short
sessions)
• AI that replies to every DM/WhatsApp/web inquiry in seconds, 24/7, and books the slot
• A reactivation campaign to your past patients in the first 2 weeks
Founding price, and if we don't beat your baseline inquiries/bookings by day 60, month 3 is
free.
15 minutes this week to see if [Clinic] is a fit? [calendar link]

Email 4 (day 14) — Subject: closing the loop
Dr. [Last name], I'll stop here so I'm not clogging your inbox. If patient follow-up or content
becomes a priority, reply "audit" any time and I'll send the 2-min video. Wishing [Clinic] a
great month.
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The 2–3 minute Loom audit

1. **0:00–0:15** 'Dr. [Name], this is Dr. Azka. Three things I found on [Clinic], in under 3 minutes.' (Their profile is on screen.)
2. **Content:** the bio vs. the 5-second rule, and the 1 post closest to converting and why. One rewritten hook.
3. **Leak:** your inquiry timestamp and their reply time. What a 30-second AI reply would have said.
4. **Database:** 'Patients due for [Botox / hygiene / laser maintenance] who haven't been messaged are usually the fastest win.'
5. **Close:** 'If useful, I'll walk you through how I'd fix all three. Link below.'

5. The sales call (30 minutes)

Minutes	What happens
0–3	Rapport, doctor to doctor. Agenda: 'I'll ask some questions, show you what I found, and if it fits, explain the pilot.'
3–15	Discovery (your File 3 questions): where new patients come from, monthly inquiries, inquiry → booking rate, after-hours handling, follow-up of non-bookers, size of the inactive database, which treatment they want to grow, past agency experience, who else decides.
15–20	**Leak map:** draw content → inquiry → response → booking → follow-up → reactivation and mark their leaks.
20–25	**ROI in their numbers** (below), then the offer.
25–30	Objections (use your File 3 answers), then close: 'Shall I send the agreement? We can start onboarding on [date].'

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ROI calculation example (always use the doctor's own numbers)
Average treatment value (e.g. filler/laser course):  $700
Typical patient value over 12 months:                 $1,800
Extra booked patients/month needed to cover $1,000:   ~1–2
Patients reactivated from database in month 1:        (their estimate)
→ "If this system books 5 extra patients a month, what does that mean for [Clinic]?"
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6. Pricing: pilot and after the pilot

A problem in your current price list

Package	USD price	UAE price	UAE in USD (÷3.67)	UAE discount vs USD
1 · Content	$1,000	AED 3,500	≈ $953	≈ 5%
2 · Content + core AI	$2,000	AED 5,000	≈ $1,361	≈ 32%
3 · Full suite	$3,000	AED 6,000	≈ $1,634	≈ 46%

The UAE upgrades are almost free (only AED 1,000 between Package 2 and 3, yet Package 3 adds an AI receptionist and 6 more systems). Dubai aesthetic and dental clinics are premium buyers. The UAE pricing needs to scale the same way as the US pricing.

Your margins at $1,000 are also too thin. 40 videos/month means 40 scripts + medical fact-checking + 40 edits. Even with an outsourced editor at an assumed ~$10–20 per short video, editing alone could be $400–800, before your own scripting time, software and revisions. $1,000 works for a *pilot*, not as your standard price.

Pilot pricing: 'Founding Clinic Partner' (60 days, 5 spots)

	USA	UAE
Monthly fee (2 months)	**$1,000/month**	**AED 3,500/month**
What's included	Package 2 value at the Package 1 price: strategy + 40 doctor-reviewed scripts + editing for 1 doctor · AI instant reply (IG + WhatsApp + web) with qualification · Booking + 48h/24h/day-of reminders · Missed-call text-back · 1 database reactivation campaign · Weekly 15-min check-in · Day-30 and Day-60 Growth Reports	
Setup fee	Waived (normally $1,500)	Waived (normally AED 5,000)
Pass-through costs	Software, phone numbers, WhatsApp Business messaging and AI usage are billed at cost, or included up to a stated cap. Put this in writing so a busy clinic can't wipe out your margin.	
What they give you	Baseline data access · 2 filming sessions (~3 hrs each) · 48h script approvals · Case-study + video testimonial permission if results are achieved · A referral if they're happy	
Risk reversal	'If we don't beat your baseline inquiries/bookings by day 60, month 3 is free.' Define the baseline in writing on day 1.	
Why paid, not free	Paying clients show up to filming, approve scripts and take it seriously. A free pilot attracts the least committed doctors and produces weak case studies.	

Standard pricing after the pilot (recommended)

Package	USA / month	UAE / month	Setup (one-time)
1 · Content Engine (1 doctor, 40 videos)	$2,000–2,500	AED 6,500–8,000	none

2 · Content + Core AI ⭐	$3,500	AED 11,000	$1,500 / AED 5,000
3 · Full Suite (AI receptionist + all systems)	$5,000	AED 16,000	$2,500 / AED 8,000
Clinic branding (4–5 specialists)	Package price + $1,200 per extra doctor	+ AED 4,000 per extra doctor	as package
AI-only (clinics that already have content)	$1,500–2,500	AED 5,000–8,000	$1,500 / AED 5,000

- **Founding clients** keep about 30% off standard for 6 months after the pilot (e.g. Package 2 at $2,450 / AED 7,700). They were your first believers.
- **Price increases go in steps:** pilot → 'early clients' (after 2 case studies, ~15% under standard) → standard. Raise prices each time your calendar fills.
- **Benchmark before you finalise:** get 3–5 quotes from Dubai and US healthcare content agencies (pretend to be a buyer or ask clients what they paid) and position Package 2 at or above mid-market. You offer content *and* AI *and* doctor review.
- **Minimum term after the pilot:** 3 months. Content compounds; month 1 isn't a fair test.

What the new pricing does to your goals

Goal	Clients needed at your old ~$1.5–2k average	Clients needed at a ~$3k blended average
$10–12k/month	6–8	4
$50k/month (Jun 2027)	25–35	~17
$100k/month (Dec 2027)	50–70	~33

Fewer clients means better delivery, better case studies and a team you can actually manage.

7. Delivery: the 60-day pilot playbook (per clinic)

Day	Milestone
0–3	**Onboarding:** kickoff call, account access, **baseline capture** (last 30–90 days: IG insights, inquiries by channel, median response time, inquiry → booking %, no-show rate, review count, inactive-patient count), brand/voice capture, and a licence and advertising-approval check.
3–7	Strategy + content calendar + first 20 scripts. **Medical fact-check** and compliance check on every script. Client approves within 48h.
7–10	**Filming session 1** (~3 hrs, 20 videos): send a shot list, outfit changes, B-roll list, teleprompter file.
7–14	**AI go-live:** instant reply on IG/WhatsApp/web, qualification, booking + reminders, missed-call text-back. Guardrails: AI identifies itself, gives no clinical advice, hands over to a human for anything medical or upset.
10–14	**Reactivation campaign** to consented past patients, segmented by treatment cycle. This is usually your first visible win.
10–60	Posting 1–2/day. Weekly 15-min check-in. Double down on any 3–5× outlier.
30	**Midpoint Growth Report** (content, acquisition and AI metrics vs. baseline). Adjust the plan.
30–35	Filming session 2 (20 videos) + next 20 scripts.
55–60	**Final report** + case study + video testimonial + renewal proposal (founding rate, 3-month term).

Metrics to show every month (from your File 3)

Content: posts, views, reach, profile visits, follows, saves/shares, keyword comments/DMs

Acquisition: new inquiries, qualified inquiries, calls, DMs, booked consultations, inquiry → booking %, new patients, estimated revenue

AI / retention: inquiries answered, median response time, follow-ups sent and recovered, reactivated patients, reactivation bookings, no-show rate, new reviews

Attribution: DM keywords per Reel, 'How did you hear about us?' at intake, a tracked booking link or number per channel

Your capacity (don't over-sell)

5 clinics × 40 videos = **200 videos/month**. Before signing client #3, line up 2–3 vetted editors (test each with a paid sample) and a scripting assistant (a medical student or junior doctor is ideal for evidence checks). You keep strategy, final medical review and client calls. Use one script template, one caption style per client, and a shared approval board.

8. Calendar: November–December 2026

Week	Focus	Target
Nov 1–7	Week 0 setup, list of 400, domains warming, first 7–10 posts of your own content, 9 sample scripts, AI demo video	Assets ready; 100 IG engagements
Nov 8–14	IG + LinkedIn outreach (Tier A), leak tests, first Looms	100 DMs · 100 LI · 15 Looms · 3–5 calls
Nov 15–21	Email goes live (Tier B), continue IG/LI, first closes	1–2 founding clinics signed
Nov 22–28	Onboard client 1–2 (baseline!), keep outreach at full volume	3 clinics signed · AI live for client 1
Nov 29–Dec 5	Last founding spots, filming sessions, reactivation campaigns	4–5 clinics signed → close the pilot
Dec 6–19	Delivery focus; post 'build in public' updates and early wins (with permission)	First reactivation results; content live
Dec 20–31	Day-30 reports for early clients; open an 'early client' waitlist at the higher price	Waitlist of 5+ clinics
Jan–Feb 2027	Day-60 reports, case studies, conversions to standard/founding rates	$10–12k MRR; 2+ published case studies

9. Competitor positioning (outreach version)

Document 1, section 2 has the full competitor table. In sales conversations, you're usually compared with three kinds of alternative:

Alternative	Their weakness	Your line
Generic social-media agency	Flyers and trends, no medical accuracy, measures likes	'Every script is written for patients and checked by a doctor, and I report bookings, not likes.'
AI / automation vendor	Tool with no demand behind it; no clinical guardrails	'AI with nothing feeding it is an empty waiting room. I bring the patients *and* catch them.'
Doctor-branding coach (Amra Farooq-style)	Teaches; the doctor still does the work; no systems after the inquiry	'Done for you, plus what happens after the DM, which is where most clinics lose patients.'
Ads agency (Joshua Navaro-style)	Paid traffic stops when spending stops; ROI claims without attribution	'Organic content builds an asset you own, and we show exactly how each patient was attributed.'

Keep verifying your competitors (monthly, 1 hour)

Keep the same sheet columns you already use (hook, format, CTA, white space). Add 3 columns: **views vs. follower count** (to spot 5× outliers), **offer/price if visible** and **market** (UAE/US/UK/India). Most of your current competitor set is India-based. Add 5 UAE-based and 5 US-based healthcare marketers so your gap analysis matches your target markets.

10. Compliance checklist (UAE + USA)

This is general guidance to raise with a local lawyer and the relevant regulator. It is not legal advice, and rules change, so check the current official guidance.

Area	UAE	USA
Medical advertising	Health advertising is regulated by the emirate's health authority (DHA in Dubai, DoH in Abu Dhabi, MOHAP elsewhere), and some content may need prior approval. Confirm what applies to each client before posting promotional content.	FTC rules on truthful advertising and endorsements; state medical/dental board advertising rules (e.g. restrictions on calling yourself a 'specialist' in non-recognised specialties such as cosmetic dentistry in many states).
Patient images & testimonials	Written patient consent; respect privacy and health-data laws; avoid misleading before/afters.	HIPAA: written patient authorization to use identifiable photos/stories in marketing. FTC: testimonials must reflect typical experiences or be clearly qualified.
Reviews	No fake reviews; respond without revealing patient details.	The FTC prohibits fake reviews and review suppression. Be careful with incentives, and never disclose PHI in replies.
Automated messages & AI calls	Use opt-in WhatsApp Business messaging; follow UAE data-protection and health-data rules on storage and transfer.	TCPA consent rules for automated texts and AI/artificial-voice calls (prior express consent; marketing needs stronger consent). HIPAA Business Associate Agreements with AI/CRM vendors that handle PHI.
Cold outreach (you)	B2B outreach with a clear opt-out; respect data-protection law.	CAN-SPAM: accurate sender, opt-out link, physical address.

11. Value per second: applied to the pilot

- **In client content:** use the 5-beat template and the 10 rules from Document 1, section 3. Each video gives the patient one answer to a real question they have before booking.
- **In outreach:** value per line. Their data (leak test) beats your credentials. Your credential (doctor) gets one line.
- **In audits:** 3 findings in under 3 minutes, each with a fix. Never a 10-minute tour.
- **In reports:** lead with 3 numbers (inquiries, bookings, reactivated patients), then the detail.

> **The one sentence that ties it all together**
>
> 'I'm a doctor. I make content that brings the right patients to your clinic, and I build the systems that make sure none of them slip through the cracks.'

Your next 7 actions

1. Fix your IG bio and LinkedIn headline (section 3).
2. Buy 2 outreach domains and start inbox warm-up today.
3. Build the list: 100 UAE + 100 US clinics this week, scored out of 10.
4. Record the AI receptionist/DM demo and write 9 sample scripts.
5. Draft the pilot agreement and have it reviewed.
6. Run 20 leak tests and record your first 10 Looms.
7. Post your first 'I messaged 10 clinics at 9 pm' video (anonymised).